COMD 4381 Final Exam Study Guide

Speech Sound Disorders in Children

Corrected study edition • November 2026

Use this guide to learn the distinctions behind assessment, development and intervention, then practice applying them in the study lab. It covers the 31 topics in the original 2025 final guide and one supported goal-writing supplement. The course packet identifies Rachel Powell and the ninth edition of Articulation and Phonological Disorders. Confirm your current instructor's scope before relying on the historical format.

What has been corrected

- The early eight include /p/. IPA /j/ means the sound in yes, not the affricate in jump.
- Ear-training isolation means detecting a target within words and longer speech.
- Multiple oppositions and maximal oppositions are different approaches. Core vocabulary targets inconsistent phonological disorder, not CAS by default.
- Speech perception training is not treatment of peripheral hearing loss, and PROMPT is not defined by an appliance inserted into the mouth.
- Developmental ages and dialect patterns require context. The old percentage claim and dose versus trials wording remain qualified rather than silently assigned a guessed key.

How to use the guide

Read each definition and example, explain the checkpoint without looking, and use the lab's aligned practice. Review the explanation after checking an answer. Original mock questions assess the same kinds of distinctions with new examples; they do not predict the next final.

The original guide announced 38 questions and 100 points. The supplied photographs show 36 questions, and the historical chat does not recover the remaining two. This corrected guide teaches the full guide scope rather than assuming that unseen material is unimportant.

1 Conversational speech samples

Connected speech shows production in naturally linked utterances, including coarticulation and prosody.

A sample can reveal variability, intelligibility, rate, stress and performance in the child's own vocabulary. A single-word test controls opportunities more efficiently; neither source describes the whole speech system alone.

Challenges include reluctance to talk, time for collection and transcription, unclear intended words when speech is highly unintelligible, and uneven sound coverage. A child may avoid difficult words. More representative everyday performance is an advantage, so it cannot be the answer to a question asking for a disadvantage.

Use engaging materials and allow the child time to respond. Record the context and intended message when possible. Add structured elicitation for sounds or positions that the sample does not contain.

Checkpoint A missing sound in a short conversation is not proof that the child cannot produce it.

Sources: ASSESS

2 Service delivery and therapy activities

Setting and activity describe different dimensions of therapy.

Push-in or classroom-based service occurs in the classroom and can connect speech goals to classroom vocabulary and participation. Pull-out service occurs outside the ordinary classroom or natural activity. Individual versus group describes session format.

Drill gives the clinician substantial control over presentation and pacing to elicit many responses. Drill play adds a game or motivating turn around the drill. Structured play builds planned speech targets into the play itself.

For example, naming five target pictures before rolling dice is drill play. Requesting toy animals with planned target words during a farm activity is structured play. Either activity could occur in a classroom or a separate room; setting does not determine activity type.

Checkpoint Ask separately: Where is therapy? Who participates? How are responses elicited?

Sources: DEL

3 Perceptual ear training

Identification, isolation, stimulation and discrimination are four different listening tasks in the traditional framework.

Identification introduces the target's auditory, visual and movement characteristics. Isolation asks the learner to detect it within words and increasingly complex speech. Stimulation supplies repeated accurate models. Discrimination asks whether a production is correct.

Listening for /s/ in a spoken word is an isolation task in this terminology; producing /s/ alone is a production task. The older filled guide confused these meanings. Perceptual support can accompany production work, but a perception task is not itself an articulation response.

Checkpoint Identify the required listener action before naming the stage.

Sources: MOTOR

4 Case history content

Case history collects background information that makes test performance interpretable.

Include the presenting concern, onset and course, birth and developmental history, hearing and medical history, family communication history, languages and dialects used, education, prior evaluation or treatment, and the communication priorities of the child and family.

Open-ended prompts such as “When is your child hardest to understand?” can expose setting and partner differences. Follow with focused questions when a detail needs clarification. Reports from caregivers and teachers complement direct sampling; they do not replace it.

Checkpoint Family history and birth history can both belong in a case history.

Sources: ASSESS

5 AAE sound patterns

African American English is a systematic dialect with context-sensitive variation.

Examples include [f] for /θ/ medially or finally, [d] for /ð/ in some initial or medial contexts, and [b] for /v/ in some contexts. Final cluster reduction, variable /r/ and /l/, and vowel differences also occur. Features are optional and vary across speakers and contexts.

Correct the old examples carefully: “test” to “tes” removes one member of a cluster, not an entire final singleton; “pin” and “pen” may merge before nasals. A dialect pattern is not automatically an error. Compare the child with their actual speech community, not an assumed dialect assigned from race.

Checkpoint Identify the phonetic context and dialect before calling a difference disordered.

Sources: VAR

6 Childhood apraxia of speech

CAS involves impaired planning or programming of the movements for speech.

The course emphasizes inconsistent consonant and vowel errors across repetitions, disrupted or lengthened transitions, and inappropriate prosody, especially stress. These features guide assessment; no one feature alone proves CAS.

CAS concerns movement planning and programming. Dysarthria concerns neuromuscular execution. A structural limitation such as a palatal difference is a different source of difficulty. Conditions may coexist, so a child’s full profile matters.

Treatment emphasizes accurate speech movement sequences, many relevant practice opportunities, cue adjustment and prosody. The goal is not merely repeated isolated oral movements unrelated to speech.

Checkpoint Inconsistent words do not automatically mean CAS; assess movement transitions and prosody too.

Sources: MOTOR

7 Common phonological patterns

A phonological pattern describes a recurring change across productions.

Final consonant deletion: “boat” → “bow,” with the final /t/ absent. Cluster reduction: “spoon” → “poon.” Weak syllable deletion: “banana” → “nana.” Reduplication: repeating a syllable, such as “water” → “wawa.” These change syllable or word shape.

Velar fronting: /k/ → /t/ in “key” → “tea.” Stopping: a fricative or affricate becomes a stop, as “sun” → “tun.” Gliding: /r/ or /l/ becomes a glide, as “red” → “wed.” Assimilation: one sound becomes more like another, as “dog” → “gog.”

Also learn depalatalization (“shoe” → “Sue”), deaffrication (/tʃ/ → /ʃ/, “chew” → “shoe”), and voicing changes (“pig” → “big”). Distinguish the feature that changed; “ship” → “sip” changes place, whereas “sip” → “tip” changes manner.

The original guide requests eight patterns, the filled guide supplies one eight-item selection, and the course assignment asks for ten. This broader set prevents omission of reduplication and other supplied examples. The instructor’s intended exact eight-item list remains unspecified.

Checkpoint Describe the sound change first, then assign its name.

Sources: DEV

8 Stimulability

Stimulability is the ability to produce an improved or correct error sound with a model and support.

Test relevant levels such as isolation, syllables and words, and record how much help was required. Successful imitation is different from spontaneous use.

A child who can say /k/ after a model may still replace it with /t/ in conversation. Stimulability informs target selection and treatment planning; it does not by itself rule out a disorder or guarantee spontaneous recovery. Contextual testing asks which surrounding sounds or positions facilitate production.

Checkpoint Imitation with support is evidence of capacity, not proof of everyday mastery.

Sources: ASSESS

9 Phonological awareness

Phonological awareness is conscious attention to and manipulation of spoken sound structure.

Tasks include rhyme, syllable segmentation, onset–rime work, blending, segmenting and manipulating phonemes. Phonemic awareness is the phoneme-level part of phonological awareness, not a completely separate prerequisite stage.

For example, blending /m/ /æ/ /p/ yields “map”; removing /s/ from “stop” yields “top.” Phonics concerns links between speech sounds and written symbols. An oral sound-manipulation task can be completed without print.

The course links phonological difficulties to risk in reading and spelling, especially when broader language difficulties coexist. Monitor literacy and language instead of assuming that every child with SSD will develop a reading disorder.

Checkpoint Identify whether the task concerns sounds, meaning, print or articulatory movement.

Sources: PHON

10 The therapy event

Antecedent, response and consequence describe what happens before, during and after a teaching opportunity.

The antecedent can include the picture, instruction, model or prompt that sets up the response. The response is the child's target behavior. The consequence follows, such as feedback or an earned game turn.

In "Look at the picture; say the word," followed by the child's production and specific feedback, keep the instruction separate from what the child actually said. Record the response so the trial contributes useful data.

A baseline probe should measure starting performance under stated conditions. Adding teaching feedback during every baseline response can blur the distinction between measurement and intervention.

Checkpoint The clinician's cue is not the child's response.

Sources: DEL

11 Dismissal and service decisions

Dismissal is a decision about continuing a particular service after reviewing progress and needs.

The historical course lists goal attainment, an allotted service period, plateau and poor motivation or follow-through as considerations. This explains the old exam's combined-inclusion answer.

For actual planning, investigate why progress has slowed: target suitability, opportunities, access, attendance, supports and treatment fit may matter. Meeting a time limit or observing poor engagement is not proof that the communication need has disappeared.

At successful completion, consider independent use and retention outside treatment as well as in-session accuracy. A change in service format or plan can be appropriate even when some needs remain.

Checkpoint Distinguish the course's list of considerations from an automatic clinical dismissal rule.

Sources: DEL

12 Morphemes and phonemes

Morpheme is the smallest unit carrying meaning or grammatical function.

A free morpheme can stand alone, such as "dog." A bound morpheme must attach, such as plural -s or past-tense -ed. "Dogs" contains the free base and the plural morpheme.

A phoneme is a contrastive sound unit. Changing /p/ to /b/ distinguishes "pat" and "bat"; /p/ itself is not the meaningful word part. Syllables organize speech but need not correspond one-to-one with morphemes.

Checkpoint Count meaning units separately from sounds or syllables.

Sources: DEV

13 Age and developmental expectations

Developmental interpretation requires the pattern the child's age and the linguistic context together.

The course describes a major decline in many simplifying patterns between ages three and four. Final consonant deletion, reduplication, assimilation, weak syllable deletion and velar fronting generally decrease substantially during early development; other patterns can persist longer.

At age two, fronting and cluster reduction alone do not establish disorder. At age four, persistent final consonant deletion together with other simplifying patterns is more concerning and warrants assessment. This explains the contrast between historical Q 11 and Q 12.

Do not convert the filled guide's broad "gone by three" or "gone by five" lists into universal diagnostic cutoffs. Sound, position, frequency, language, dialect, intelligibility and the reference sample affect interpretation. A single example is not a complete diagnosis.

Checkpoint Age makes the same production more or less concerning; it does not replace assessment.

Sources: DEV

14 Early middle and late eight sounds

Shriberg's sound groups summarize relative acquisition order in the course framework.

Early eight: /p, b, m, d, n, w, j, h/. Here /j/ is the "y" sound in "yes." The old filled guide accidentally omitted /p/.

Middle eight: /t, ŋ, k, g, f, v, tʃ, dʒ/. These include "ng," "ch," and the "j" sound in "jump." Late eight: /ʃ, ʒ, s, z, θ, ð, l, r/.

The group labels are not a guarantee that every individual child acquires every member at one fixed birthday. They are also not an independent cluster inventory; clusters require combinations of sounds.

Checkpoint Distinguish IPA /j/ in yes from /dʒ/ in jump.

Sources: DEV

15 Dodd classification

Phonological delay is the largest subgroup in the course's cited classification.

Articulation disorder involves consistent difficulty with a limited set of sounds, often distortions. Phonological delay involves developmental patterns persisting beyond expectations. Consistent phonological disorder includes stable atypical or nondevelopmental patterns.

Inconsistent phonological disorder involves variable word productions without the full motor-speech profile needed for CAS. CAS concerns speech movement planning and programming. These labels help describe a profile and choose further assessment, not simply count errors.

The slide reports phonological delay at 57.5% in its referenced classification. Treat that as the course's specific cited distribution, not a universal percentage of all children with SSD.

Checkpoint Consistency and developmental appropriateness answer different classification questions.

Sources: DEV

16 Prompts and cues

Prompt versus cue is direct production support versus a less explicit hint in this course's convention.

A prompt may give a model or explicit placement instruction. A cue may remind the child to use a learned skill without specifying exactly how to produce it. "Put your tongue behind your teeth" is more direct than "Remember your sound."

Terminology varies across approaches; a gesture may function as either depending on how much information it supplies. Describe the support and level of independence instead of assuming that all visual input has one label.

Checkpoint Judge the information given, not merely whether the support is spoken or visual.

Sources: DEL

17 Hearing screening

Hearing status must be considered early when interpreting speech performance.

The course sequence places hearing screening before the main articulation and phonological testing. A recent appropriate hearing evaluation may already supply relevant information; a failed screen requires follow-up rather than a guessed explanation.

Difficulty hearing speech contrasts and difficulty planning speech movements are different possibilities. Do not assume that a child's failure to produce a sound establishes either one.

Checkpoint A screen identifies the need for further evaluation; it is not a complete audiological diagnosis.

Sources: ASSESS

18 Orofacial examination

Gloves, light and tongue blade are the basic tools listed in the course notes.

The examination considers structure and function relevant to speech: lips, tongue, palate, dentition and occlusion, symmetry, range and coordination. Findings help determine whether further assessment or referral is needed.

Tools enable observation; their presence does not establish a diagnosis. Avoid the older assignment's implication that a structural finding automatically calls for surgery or that surgery necessarily resolves speech errors.

Checkpoint For the historical written question, name at least two actual tools.

Sources: ASSESS

19 Assessment and diagnosis

Assessment is the evidence-gathering and interpretation process.

Diagnosis is the resulting judgment about the presence, absence and nature of a disorder. Differential diagnosis distinguishes between plausible types or causes that can produce similar surface signs.

Use converging information from history, hearing, oral structures, single words, connected speech, patterns and stimulability. A score can contribute evidence; one low score does not independently explain the cause.

Checkpoint Data collection and the judgment made from those data are related but distinct.

Sources: ASSESS

20 Long term goals and short term objectives

Long term goals describe meaningful communication outcomes; objectives define steps toward them.

A long-term goal can target intelligible production in functional communication. Short-term objectives specify smaller skills or conditions that support the outcome, such as a target in words, then sentences and conversation.

An objective should not merely repeat the long-term goal with a different label. State what performance is expected, under which conditions and with what criterion. Link each step to a real communication need.

Checkpoint Identify the destination and the measurable step toward it.

Sources: GOAL

21 SMART goals

SMART means specific, measurable, achievable, relevant and time-bound.

Specific names the target behavior. Measurable makes performance observable. Achievable relates the demand to the learner's baseline and available support. Relevant connects the behavior to communication needs. Time-bound establishes when progress will be evaluated.

Illustrative goal: Within eight weeks, during picture description with no more than one reminder, the child will produce initial /k/ in at least 16 of 20 opportunities across three probes. The numbers are an example, not a recommended prescription for every child.

An accuracy criterion alone does not supply a timeline. "Over three probes" states stability across measurement opportunities; "within eight weeks" supplies a deadline.

Checkpoint A measurable goal still needs an appropriate baseline and a meaningful purpose.

Sources: GOAL

22 Generalization and maintenance

Generalization is transfer to untrained material or contexts; maintenance is retention over time.

Generalization can cross words, word positions, linguistic levels, sounds or settings. After training /s/ in initial words, improvement in untrained final words is a transfer result. Improvement in conversation outside the clinic is another.

Maintenance asks whether gains persist later, especially when intensive teaching and prompts have decreased. A follow-up sample several weeks later answers a different question from same-day production of new words.

Use untrained probes to distinguish learning the practiced list from broader change. Accuracy in a drill session alone demonstrates neither full generalization nor long-term maintenance.

Checkpoint New context points to generalization; persistence across time points to maintenance.

Sources: GOAL

23 Dialect and disorder

Dialect is a rule-governed variety of a language.

Regional, social and community varieties have systematic patterns across sounds and other language domains. A difference from mainstream classroom English does not itself establish disorder.

A dialect speaker can also have an SSD. Assess whether difficulty is unexpected within the person's language and dialect systems and affects communication. A culturally responsive assessment neither labels every difference a disorder nor assumes dialect rules out disability.

Checkpoint Difference and disorder can coexist; one does not establish or exclude the other.

Sources: VAR

24 Baseline measurement

Baseline describes performance before the target is taught.

State the target, elicitation conditions, level of support and scoring rule. These make later comparisons interpretable. For example, record correct productions out of a specified number of opportunities before instruction starts.

A probe can be used at baseline or later to monitor change. The word "baseline" identifies when and why the measure is collected, not a unique test format. Repeatedly practicing every probe word can compromise its value as evidence of transfer.

Checkpoint Compare like conditions or explicitly explain changes in support.

Sources: DEL

25 Parent interview and case history

Caregiver interview adds developmental and everyday communication information.

Ask about who understands the child, settings that are difficult, early speech development, language exposure, medical and hearing history, prior services and family priorities. Clarify differences between partners or settings.

A case history can combine written questionnaires, interviews and records. It is not synonymous with the entire comprehensive speech evaluation. Reports about perceived intelligibility complement direct intelligibility measurement.

Checkpoint History explains context; direct sampling describes observed performance.

Sources: ASSESS

26 Intervention comparison

Treatment selection connects a proposed mechanism to the observed speech profile.

Traditional articulation: establish a target sound, stabilize it through increasing linguistic complexity, transfer it and maintain it. The course's five phases are perceptual training, establishment, stabilization, transfer and maintenance. Motor practice may use imitation, phonetic placement, shaping or facilitating contexts.

Minimal pairs: use words differing by one phoneme to establish a meaningful contrast, such as "tea" and "key" for a child who fronts /k/. Minimal oppositions use a relatively small feature contrast. Maximal oppositions select sounds differing along several dimensions. A minimal pair differs by one phoneme; that does not mean the phonemes differ by only one feature.

Multiple oppositions: contrast one substitute with several intended sounds in an integrated set when several phonemes collapse to one output. The aim is to reorganize a broad loss of contrasts. It is not another name for maximal oppositions.

Complexity: select structurally or linguistically complex targets to encourage change in untreated simpler structures. Course examples include marked or nonstimulable clusters. Do not reduce the approach to always selecting whatever looks hardest without examining the child's system.

Cycles: rotate through deficient phonological patterns, combining focused auditory input and production practice. The intent is emergence of patterns and improved intelligibility; mastery of one target is not required before moving to the next cycle target. The old guide's fixed session totals and strict intelligibility cutoff are not universal requirements.

Core vocabulary: stabilize functional whole-word productions for inconsistent phonological disorder. The course uses approximately 50–70 meaningful words, working on a subset and retaining inconsistent words. Consistency comes before perfect accuracy. The CAS deck explicitly separates this population from CAS.

Speech perception training: improve recognition or differentiation of speech targets and contrasts, often alongside production. This is not treatment of a peripheral hearing loss. Perception and production should each be assessed.

Psycholinguistic approach: investigate where speech processing breaks down, such as input, representations, retrieval or output, and target the supported difficulty. It is a framework for linking a processing hypothesis to assessment and intervention rather than one fixed sound drill.

Enhanced milieu teaching: arrange meaningful interactions, follow the child's interests, model language and use prompts or expansions to create communication opportunities. Naturalistic speech work can be embedded in play. It is not restricted to autism, and it does not mean unplanned play with no communication targets.

Nonlinear phonology: consider the hierarchy linking features and segments with syllables, word shapes and stress. A target may change a word's structure or prosody as well as an individual consonant. The course contrasts this with focusing only on suppression of a single surface process.

Phonological awareness intervention: explicitly teach analysis and manipulation of sound structure, with appropriate links to letters and speech goals. Phonemic awareness is not guaranteed to appear merely because a child practices rhyme. Choose tasks based on assessed skill.

Dynamic Temporal and Tactile Cueing: a motor-based approach for CAS that adjusts support as the child practices speech movement sequences. The course describes simultaneous production, direct imitation, delayed imitation and spontaneous production with flexible cueing. PROMPT is a separate tactile-kinesthetic approach; it is not defined by inserting devices into the mouth.

Checkpoint Choose the mechanism: contrast meanings, stabilize words, rotate patterns, promote complexity-based transfer, or practice speech movements.

Sources: PHON, MOTOR, PSYCH

27 Tongue thrust

Tongue thrust involves forward tongue placement or movement against or between the teeth.

It may appear during swallowing and may coexist with speech or resting-posture differences. Observe the actual function and context rather than assuming all interdental sounds share the same cause.

Tongue-tie refers to a restrictive lingual frenulum; it is not a synonym for thrust. Malocclusion describes dental relationships. These distinctions prevent selecting an anatomically related but incorrect label.

Checkpoint Identify the movement or posture described before naming the condition.

Sources: ASHA-OMD

28 Language and literacy risk

The denominator matters when interpreting a percentage about SSD and other difficulties.

The old guide states 25% for subsequent language deficits and possible literacy issues without an adequate source. The photographed exam instead asks about comorbid language disorder, and the chat answers 70% without evidence. These are not interchangeable outcomes.

ASHA summarizes about 40% concomitant language impairment among children with SSD and separately about 25% reading-service eligibility among children receiving school speech services. These figures describe different populations and outcomes. Neither is a universal prediction for an individual child.

The supported course lesson is to monitor language, phonological awareness, reading and spelling, particularly when difficulties coexist. Learn the relationship and qualify statistics with the population and outcome. The exact intended historical percentage key remains unconfirmed.

Checkpoint Do not replace a vague unsupported number with a different number stripped of its context.

Unresolved historical detail The original course percentage remains unresolved; it is excluded from automatic historical scoring.

Sources: PHON, ASHA-SSD

29 Treatment terminology

Frequency, duration, dose and trials describe different aspects of the treatment plan.

Frequency: how often sessions occur, such as twice weekly. Session duration: how long a session lasts. Target selection: what sounds, patterns or skills are addressed. Session format: individual or group organization.

A trial is a discrete teaching or response opportunity. Dose commonly counts teaching episodes within a session; describe whether you counted offered opportunities, attempted productions or correct productions. They are not automatically the same number.

A reinforcer is a consequence that increases the future likelihood of the behavior. A sticker is only a presumed reward until its effect on behavior supports calling it reinforcement.

The old exam's matching descriptions for dose and trials overlap, and the old chat invents a different row. This lab teaches operational definitions and does not grade the ambiguous historical matching block. Do not memorize "dose is opportunities, trials are total sessions"; total sessions is a different measure.

Checkpoint Write the unit with the number: sessions per week, minutes per session, or attempts per session.

Unresolved historical detail Historical Q 34 remains unscored because two descriptions do not distinguish the terms reliably.

Sources: DEL

30 Velopharyngeal insufficiency

VPI is inadequate closure associated with a structural limitation of the velopharyngeal mechanism.

Course features include hypernasality, nasal air emission and reduced oral pressure for pressure consonants. Hypernasality concerns resonance, especially on vowels and voiced oral sounds; nasal emission concerns airflow during consonant production.

Stops, fricatives and affricates depend on oral air pressure. Structural leakage may weaken them. Learned compensatory articulations can coexist and require different management from the structural problem.

Behavioral speech therapy cannot correct the structural cause of resonance difficulty. Appropriate specialist assessment determines management; speech work can address learned errors. This topic remains in the guide even though it is not visible in the supplied portion of the old final.

Checkpoint Separate structural insufficiency, resonance and learned articulation effects.

Sources: VAR, ASHA-VPI

31 A comprehensive SSD assessment

Converging evidence is more informative than any isolated task.

Include case history and interview; hearing screening or relevant hearing results; oral mechanism examination; single-word production; connected speech; pattern and intelligibility analysis;

stimulability; and contextual testing. Evaluate language, phonological awareness or other domains when indicated.

Standardized tests can compare performance with an appropriate norm group. Connected speech supplies a different context. Stimulability probes capacity with support; contextual testing locates facilitative environments; history supplies development and everyday impact.

Interpret all results in light of languages, dialects, age and the suitability of the procedures. A norm-referenced score has limits when the norm group or administration does not fit the child. Synthesize the findings before deciding whether a disorder is present and how to respond.

Checkpoint Match each assessment component to the question it answers.

Sources: ASSESS

Supplement 32 Goal writing components

Performance, condition and criterion make an objective observable and interpretable.

Performance names what the learner will do, including the target and linguistic level. Condition specifies the context, materials or support. Criterion states how well or how consistently the behavior must occur.

In “During picture description with one reminder, produce initial /k/ correctly in 16 of 20 opportunities across three probes,” picture description and one reminder are conditions; producing initial /k/ is performance; 16 of 20 across three probes is the criterion. Add a meaningful timeline and rationale to support the full SMART goal.

This item appears in the filled guide as 32 but is additional to the original 31-topic PDF. It is retained because the treatment-plan slides support it.

Checkpoint Do not confuse the support allowed with the accuracy required.

Sources: GOAL

Source notes

Course sources govern the teaching where available. Student notes and the filled guide were cross-checked; the predicted practice appended to the old guide was not treated as the real final. The eighth-edition textbook is identified only as a supplemental check for ear-training and dialect descriptions; it does not replace the assigned ninth edition.

ASSESS

Course Ch 6 assessment assignment; Ch 7 Assessment Decision Making, slides 12 and 18–20; Wk 04 oral examination class notes. Student notes are corroborated where needed; they are not an official key.

DEV

Course Ch 3 Part 2 Speech Sound Acquisition, slides 14, 17, 22, 26; Ch 5b CAS and Functional SSDs, slide 23; Wk 07 pattern assignment.

VAR

Course Ch 14 Language and Dialect Variation, slides 10–15; Ch 5 Organic SSDs, slides 5–10. Supplemental Bernthal, Bankson and Flipsen, eighth edition, p 281, Table 11.2 and adjacent discussion. The assigned 2025 text was the ninth edition.

DEL

Course Ch 9 Remediation Basics, slides 10–20, 24; Treatment Plans and Service Delivery, slides 3, 26, 32–40; Wk 09 terminology and Wk 10 prompts/cues assignments.

GOAL

Course Treatment Plans and Service Delivery, slides 11–17, 39–40; Wk 12 SMART assignment; Ch 9 Remediation Basics, slides 18–20.

MOTOR

Course Ch 10 Motor Based Treatment, slides 5–13, 18–21; Ch 12 CAS, slides 6–10; Ch 5b CAS and Functional SSDs, slides 5–8. Supplemental Bernthal, Bankson and Flipsen, eighth edition, p 229 for ear-training terminology.

PHON

Course Ch 11 Linguistically Based Approaches, slides 3–10, 15–23, 29–31; Ch 7 Assessment Decision Making, slides 29–30; Ch 3 Part 1, slide 17; Ch 13 Phonological Awareness, slides 3–6, 15–17.

ASHA SSD

ASHA Speech Sound Disorders Practice Portal, Incidence and Prevalence and Treatment. Accessed November 11, 2026. https://www.asha.org/practice-portal/clinical-topics/articulation-and-phonology/

ASHA OMD

ASHA Orofacial Myofunctional Disorders Practice Portal, Overview and Signs. Accessed November 11, 2026. https://www.asha.org/practice-portal/clinical-topics/orofacial-myofunctional-disorders/

ASHA VPI

ASHA Resonance Disorders Practice Portal, Overview and Treatment. Accessed November 11, 2026. https://www.asha.org/practice-portal/clinical-topics/resonance-disorders/

PSYCH

Terband, Maassen and Maas, A Psycholinguistic Framework for Diagnosis and Treatment Planning of Developmental Speech Disorders (2019), https://pmc.ncbi.nlm.nih.gov/articles/PMC7050667/; corroborates course Ch 5b slide 22.

Remaining historical uncertainties

Question 23 Timer obscures the stem; no restored wording found in the shared chat.

Question 33 The intended percentage is not verified. The old guide, screenshot selection and chat refer to inconsistent figures or outcomes.

Question 34 Dose and trials descriptions overlap. The chat substituted a different row; no official key is available.

Question 37 Not present in the supplied screenshots or shared chat.

Question 38 Not present in the supplied screenshots or shared chat.

Historical practice adaptations are labeled by their original number. The lab’s written checklists are practice rubrics, not the instructor’s official scoring guide. First answers, later corrections and self-reviewed writing remain separate.